“Introducing an App for Navigating Cancer Care” (Berries) [#67]

Brad Powe
August 9, 2023

“Our goal … is to just make patients' lives easier and give them more clarity, and let them focus on healing, while we deal with the rest.” – Yuval Gal

“We aim to be the one-stop shop for cancer patients, addressing whatever they need.” – Yuval Gal

“Where are the areas that you felt like you would love to get some help with?” - Yuval Gal

Meeting Summary

From their initial diagnosis, cancer patients and their caregivers embark on a journey with many tasks they need to navigate and manage, such as paying bills, negotiating insurance, choosing the best tests and treatments, and handling many things they are doing for the first time.

Berries is an AI healthcare start-up, offering an app designed to simplify cancer patients' recovery journey. Their web-based platform records doctor’s appointments, auto-creates medical tasks, and centralizes medical paperwork. They aim to be a one-stop platform that manages, guides, and automates cancer patients' journeys and allows them to focus on their health while Berries handles their administrative tasks.

Berries CEO Yuval Gal and co-founders Tal Salman (COO) and Yam Catzenelson (CTO) discussed the first iteration (Minimum Viable Product) of their app, including key features such as recording what to do, translating voice to text, providing a dashboard of tasks you need to do with all of your appointments, and storing medical files. They asked for feedback and guidance from the participants on problems they have and what services and features would be valuable.

Most of the discussion was on alternative personas (customer segments) that Berries might serve and their unique needs. For example,

- Less engaged patients. (The Cancer Patient Lab community is unusually highly engaged and educated.)
- Caregivers. (Many patients are tired and preoccupied, so a family member or friend, often a female spouse in the case of prostate cancer, is guiding their care.)
- Patients at the end of life. (For example, services for planning for your family members, whoever is going to live on, beyond your demise, such as a will, funeral services, or communicating with friends and family.)
- Newly diagnosed patients. (The patient's attitude is going to be massively influenced by what they know. A newly diagnosed patient may be overwhelmed and not know where to turn. They may need help in getting second opinions or finding clinical trials.)

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If you are interested in trying their app, Berries is offering you free access in return for your feedback. Your experiences and comments will guide their product improvements, making life easier for you and future patients. If you would like to schedule a call with them, the link is here.

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Meeting Notes

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Discussion Outline

1. Introduction to today's session.
2. What kind of services do you provide for end of life planning? (8:57)
3. How does the operating model work for the company? (13:04)
4. Patient attitude. (16:53)
5. What is unique about your perspective on care? (21:33)
6. How long term care insurance works. (24:09)
7. The unique characteristics of the American reimbursement system. (31:56)
8. How do you help people navigate their treatment decisions? (37:16)
9. How do you find out more about the services? (43:22)

SUMMARY KEYWORDS

patients, treatment, prostate cancer, cancer patients, clinical trials, active surveillance, question, services, people, navigate, berries, years, journey, caregiver, adt, cancer, care, friends, targeting, diagnosed

SPEAKERS

Yuval Gal (25%), Jeff Dwyer (16%), Brian McCloskey (12%), Brad Power (12%), Richard Anders (11%), Paul Van Camp (11%), Noel Resch (4%), Yam Catzenelson (4%), Robert Gurmankin (3%), Rick Stanton (2%), Tal Salman (<1%)

Meeting Transcript

Brad Power 0:03
This is the Cancer Patient Lab/Prostate Cancer Lab.
We have a special session today with Tal and Yuval of Berries, an Israeli startup that's developing apps and services for cancer patients. They're interested in getting some feedback from us today.

Yuval Gal 1:14
Yam is also part of the founding team here with us. It's a pleasure meeting you all. And I'm very excited to be here to share our vision and to learn from your experience.

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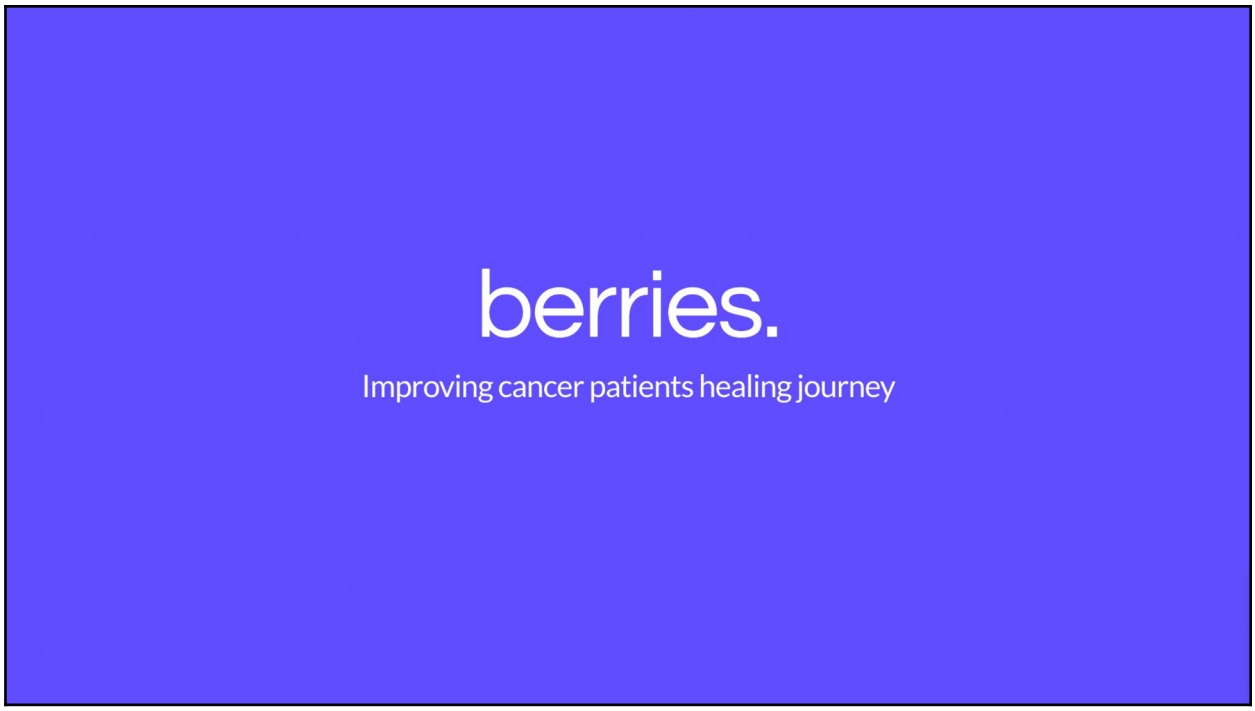

Yuval Gal 1:49
have a little intro, we'll tell you about ourselves a little bit and why we're about to do what we're doing. And then we'll just be happy to have an open discussion and answer your questions and learn from you. So at the berries, we are aiming to improve cancer patients' healing journey.

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It all started when my dad had prostate cancer, actually. Now he's fine after treatment, it seems like it worked, and he's recovering. I saw how hard it is to be a cancer patient, but not just because of the obvious reason that you're sick and unwell. But also because of the fact that you need to run this whole operation. And you need to navigate your way through this journey, and you need to handle a lot of stuff that you don't even know about when you start. It’s a lot of uncertainty and while there are great resources for benefits or things that you can get but patients don't know about them. In my case, it became my mom's full time job. Which kept her very busy.

With those in mind - we thought we could build an AI tool that will help people manage their recovery journey, guide them through their recovery journey, and also automate tasks for them. All in order to let them focus on healing. That’s what we aim to do.

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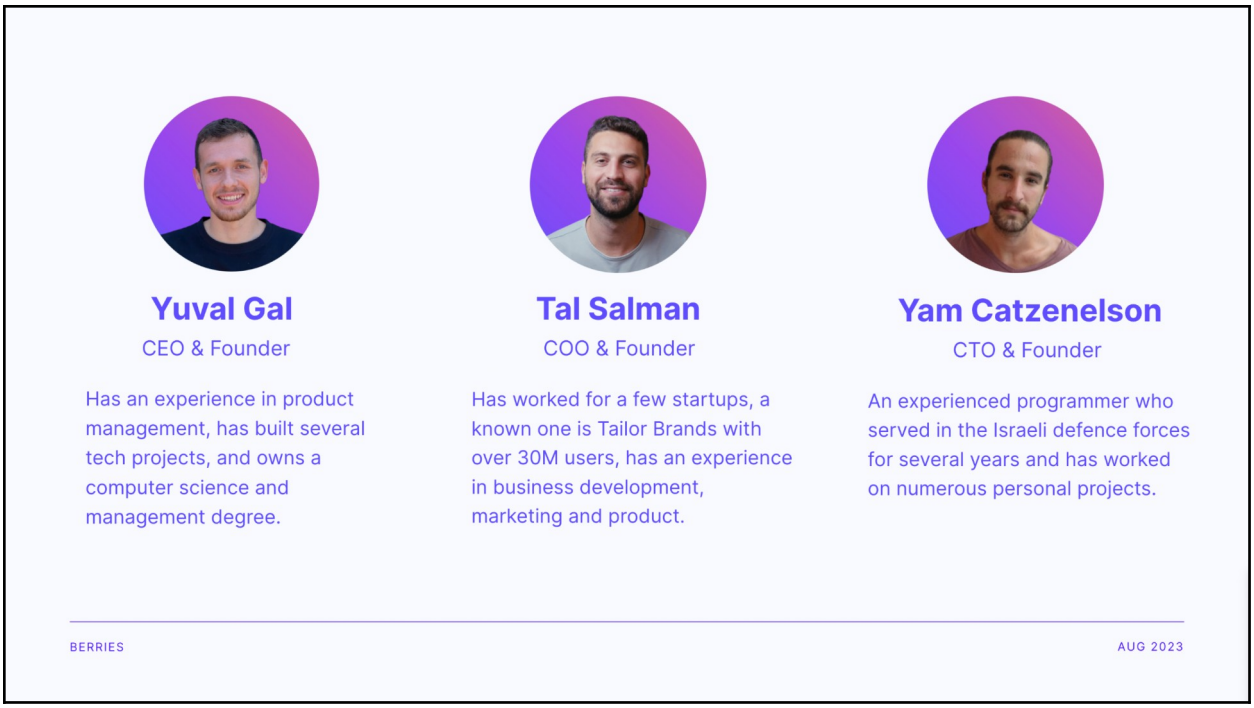
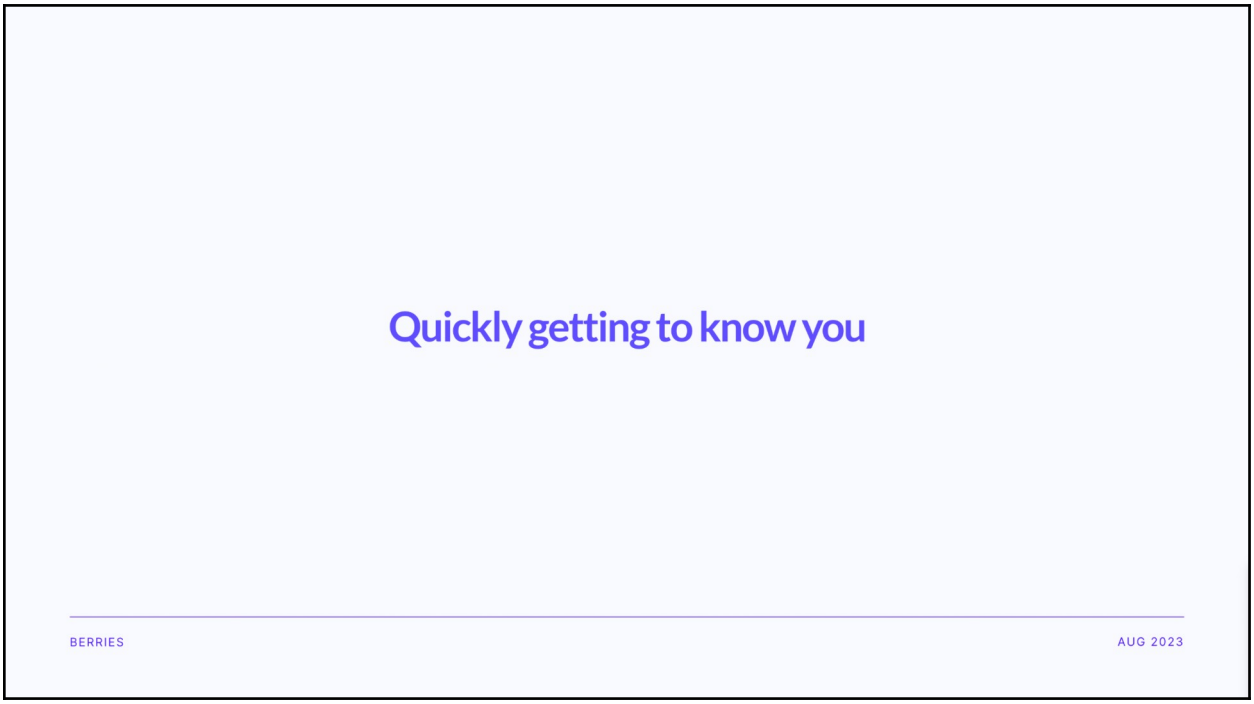


Few details about ourselves. Our team is experienced in the Israeli high tech industry. I'm the CEO.

Tal Salman 3:50
I'm the COO. It's a pleasure to be here with you guys.



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Yam Catzenelson 3:53
I'm the CTO and thank you for your time. I'll take it from here. We would like to ask you three simple questions to get to know a bit about your problems.

Raise your hand if you felt overwhelmed or uncertain about the next step in your cancer journey.

2

Raise your hand if you experienced financial stress or felt unsure about the financial benefits available during this challenging time

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Yam Catzenelson 4:37
Raise your hand if you experienced financial stress or felt unsure about the financial benefits available during this challenging time.

3

Raise your hand if you felt too drained or unwell to navigate paperwork, seek out benefits, or optimize your journey

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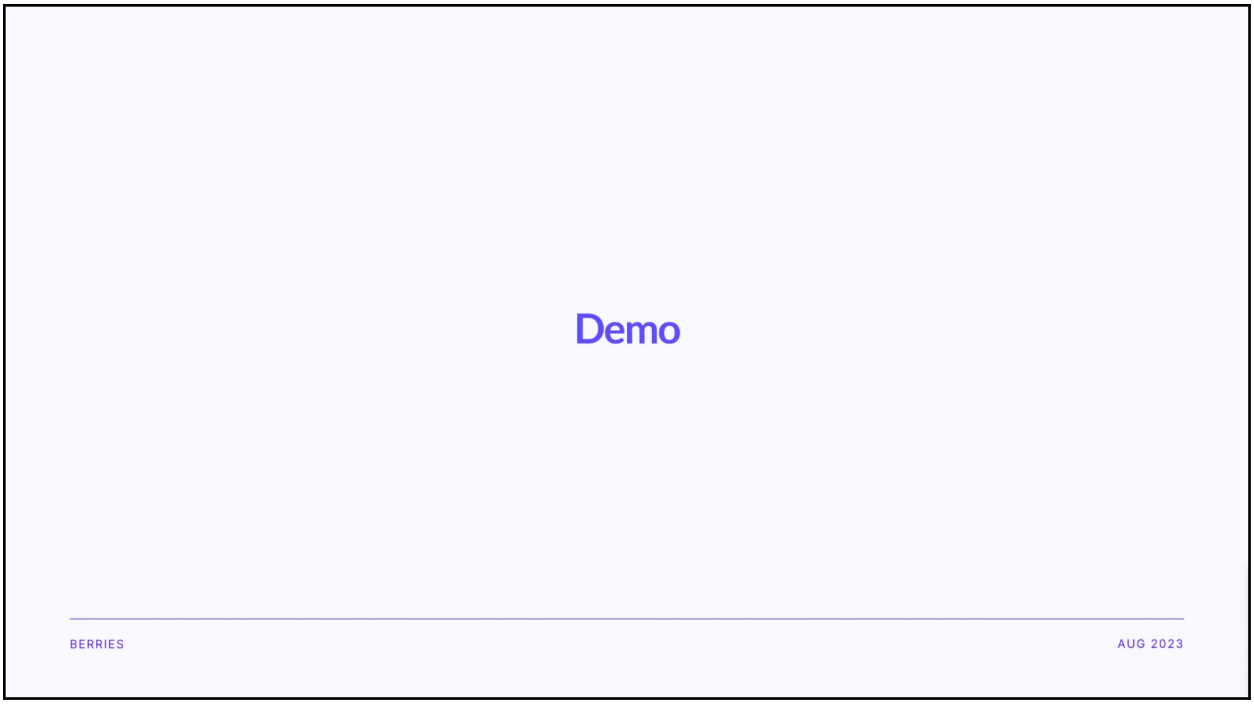
Yam Catzenelson 4:49
Last one. Raise your hand if you felt too drained or unwell to navigate paperwork, seek out benefits, and optimize your journey.

Yam Catzenelson 5:10
No one. Okay.



Yuval Gal 5:29
Yeah, some of the participants raised their hand, some not, and it's totally fine. So what we're about to do, and what we are doing in this stage now is start supporting patients, we try to give more clarity in their journey, with creating personalized guidance for them. And now I’m going to share with you our workflow.

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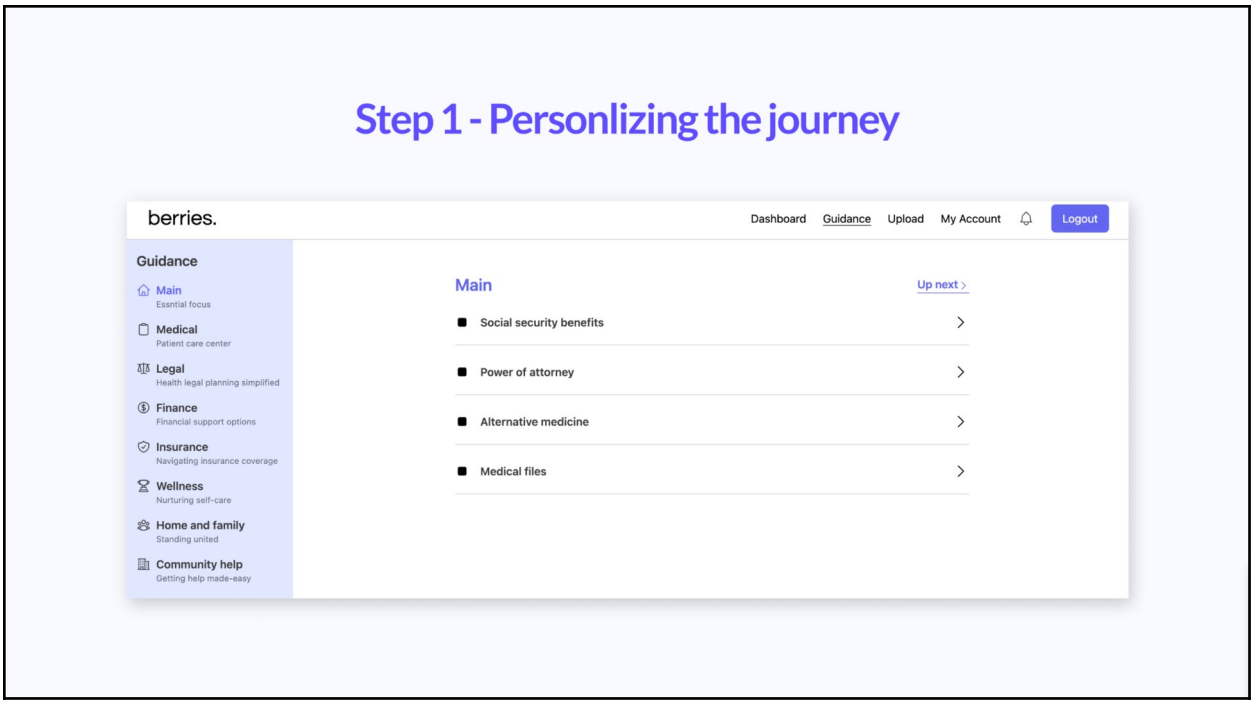
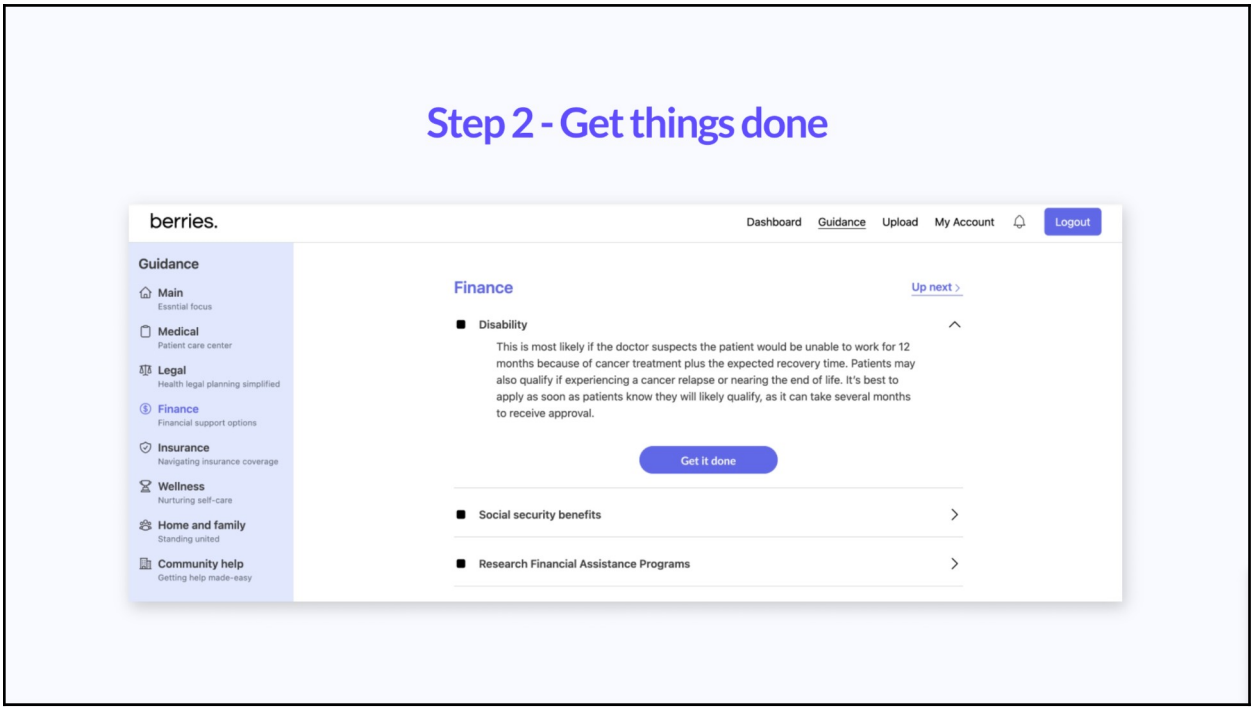
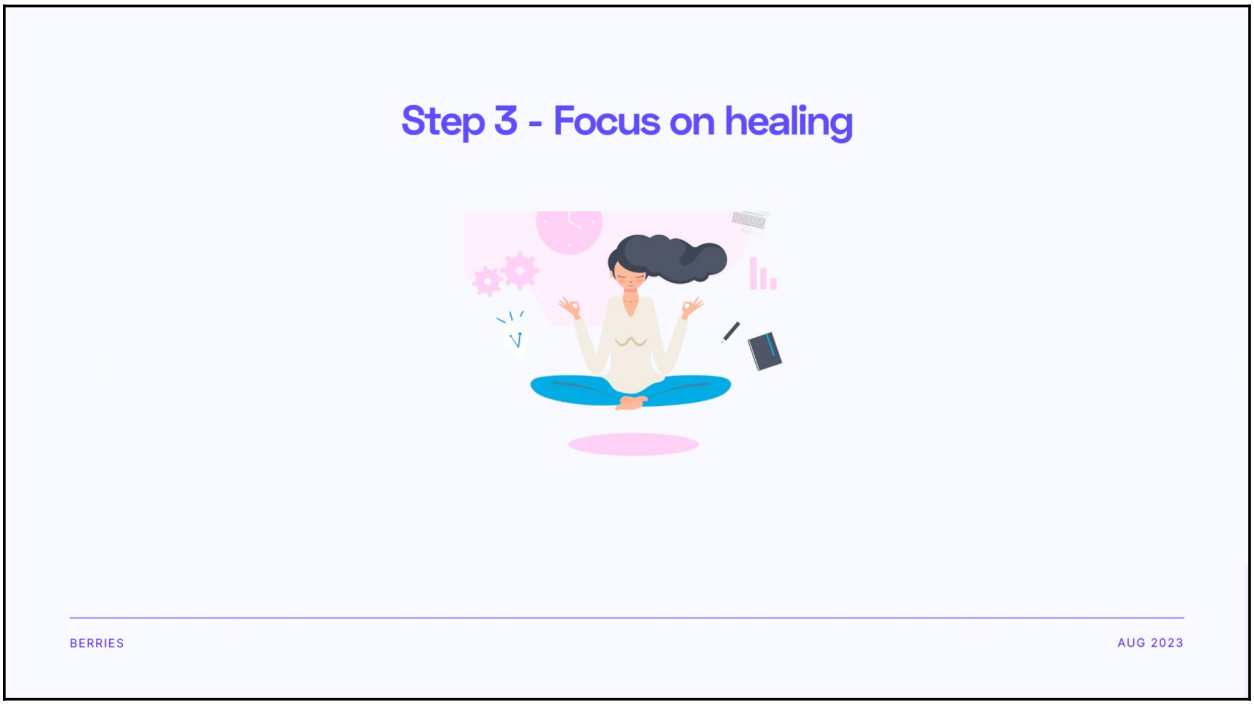


We have a welcome meeting, we understand their situation-specific needs, and mapping what they already went through and what is expected to come in the future. We create guidance in all kinds of topics like legal finance, insurance, esc. After that, patients get access to the platform, and see everything there.



The patients are able to get our help in every task they have by our care team. Our goal, as I said, at the beginning is to just make patients' lives easier and give them more clarity and let them focus on healing while we deal with the rest. In terms of pricing, the platform and the personalized guidance is for free but the part of actually executing the task for the patients is not, the price depends on the specific case and the specific task, we are getting into details in the welcome meeting.



We also provide a dashboard where patients can manage all of their tasks and a one repository of all of the medical files in order to arrange everything in one place.

That's it. We want to let patients focus on healing. They have plenty of things to deal with. And we would love to just take the load out of their shoulders off their shoulders, and handle it for them. That's our vision, and we would love to answer some questions and learn more from your experience. Where are the areas that you felt like you would love to get some help with?

Brian McCloskey 8:57
We've had a few patients recently who were sort of in the end of life stage. We're all advanced prostate cancer patients stage four, where a cure is hoped for, but it's not there. What kind of services do you provide for planning for your family members, whoever is going to live on, beyond our demise? Probably that's not the best question to answer. Ask it right out of the gate, but it's real. We recently lost a couple of folks. And so it's definitely top of mind for me.

Yuval Gal 10:10
Definitely, that's a great question. So as part of our guidance, we try to help and guide in all of the aspects that patients care about. It's definitely one of the things that come up. When we have meetings with patients. We try to provide with some legal help, like helping create a will or a living will, in order to set it up. But when asking the question you mean, if our tool can be helpful for people, for the families to handle with all of the load?

Brian McCloskey 11:02
I'll be a little bit more tactical. We had one patient reach out regarding funeral services. Things are getting tough. I need help with funeral services, and then even, celebrations of life for those individuals. All those things fall upon the caregivers in their family. If you've got young kids, for example, that maybe have even reached their college years or in college, there's the whole

financial component of being able to support them. You mentioned the will. I'm sure that there are a lot of patients that need help with that. So those are the types of things. Some that can be very tactical. But nobody's really thinking about those things, until the bitter end.

Brad Power 12:12
I want to interject quickly for the Berries team to look at: we had a session run by one of our patients talking about Caring Bridge and other online communication tools. What I heard, and some of what Brian was just saying is, you can have an ongoing conversation with a set of friends and family who care about you and want to know what's going on. And then I could imagine, as you reach the end of life, you could be having almost prepackaged messages. This is the message I want to send out after I pass or something like that. Anyway, the notion that you have a caring community that you have pre-wired would be a feature that would be valuable to people.

Yuval Gal 13:04
These are great questions. I want to stress one thing: we aim to be the one-stop shop for cancer patients, addressing whatever they need. If we determine that they require assistance in dealing with these types of challenges, we will be ready to help by either taking the load off their shoulders or providing guidance in those specific areas. That's exactly our purpose. Also, it’s important to mention that we're here to learn from your experience and consider the valuable insights you provide.

Brian McCloskey 13:45
So let's just kind of dig into that a little bit more, if you can maybe talk a little bit about your operating model. So the way that we operate is almost like a crowdsourcing way where we have strategic partners that help us to provide analytic services, for example, or diagnostic services. So, do you provide all of your resources, let's say, a lawyer? Are they in house? Do you farm that out? How does your model work?

Yuval Gal 14:20
That's also a good question. It's contingent on the tasks at hand. We have an in-house care team that might be equipped to manage this. And when necessary, we'll connect with external sources or outsource the request. It's all dependent on the specific nature of the request. To be honest, there's a possibility that we might not be able to handle certain requests right now, simply because they fall outside our scope. We're in the early stages, and we're striving to assist wherever possible. However, there are some aspects that might be beyond our capacity to help with.

Brian McCloskey 15:04
Maybe that creates partnership opportunities for us where we're working with certain groups, individuals or companies, that provide some of the services that our patients need. We could talk about that.

Brad Power 15:19

I just want to interject, Brian, my characterization of Berries, where they are now. They have some ideas, and they have built some functionality, but mostly, they are so early that it's not like asking questions, “Do you have that service?” It's more like asking us, ”What service would you like?” And then if everybody raised their hand and said, “We want that,” then they would go build it. They're at that early, Product-Market Fit stage, where they're building things, and they're listening to us today, to figure out what would be valuable.

So just to underline what you said, it's not only the newly diagnosed patient, but it could be the patient who is at the end of life. That's a different persona, that might have a different set of needs. And they might say, “Oh, we're going to be focusing more on people who are just diagnosed. Not late stage.” But that's the kind of choice that they have to make. Or, if they're covering the full lifecycle, then they need to have both.

Brad Power 16:16
They didn't give a long demo of their services. It's more like they are listening for what patients and caregivers are trying to do, and what services might be valuable.

Richard Wassersug 16:37
I have a very broad overview of what you're up against.

First of all, you noticed that much less of the hands came up when you asked, “Do you feel overwhelmed?” You're talking to a very select group of people who are really interested and deeply engaged. This is not a cross section of the patient population. You can’t make any use of that small survey from this small sample here.

Second, it's neat that you have your core team here. When you presented your own personal story about your parents, you presented evidence that shows that you're probably talking to the wrong audience. You pointed out how overwhelmed your mother was, and how she handled all this. So notice that there are no women on the call. You've got aggressive men, who are really high powered, in industry, an MD, a dentist, or whatever. This is not your sample. I do a lot of work in the psychology of men and masculinity and so forth. Typically, men will use denial as an effective way to deal with problems they cannot solve. I see some guys nodding, “Yes.” So in that situation, there's a lot of research, and it's not mine, that the partners, typically mostly female partners, step in. So often the folks who are trying to figure out what are we going to do about a funeral, if we're going to go to that point, that's not going to be the patient. They don't even know what's happening to them. They're in such bad shape at that point. There's nothing about your approach that mentioned how you would strategically go after the partners. And if you think that is worth exploring, and I'd be happy to have a longer open discussion with you, or the three of you to strategize, because that is the problem. There's so much evidence to show that they, the guys, in every way depend on their partners. Working so closely with so many patients over these years, I've dealt with hundreds of prostate cancer patients on hormonal therapy, I will not deny a denier the right to deny. That is the real psychological reality of it. If you've got to make this work, you have to reach the partners.

And then I will add one last thing, the patient's attitude is going to be massively influenced by what they know. You mentioned your dad had prostate cancer. If somebody knows nobody who's had prostate cancer, they're going to have one perspective. Somebody who has had three friends and relatives and fathers brothers, whatever, die; they're going to have a wholly different perspective. And in that sense, you can't just come up with a general approach to everyone. Look at the question about putting out a will. I have a will, actually. But I mean, that's not my position. So you're going to have to find the right people at the right time and approach them in the right way. I can say that's also true for ADT. If you try to tell someone about the ADT, managing the side effects of hormonal therapy that is present on active surveillance, it's meaningless to try to talk to a patient who's already diabetic, massively depressed, overweight from these drugs. It's too late. So you have to reach the right patient. It's not the right population, it's going to be the women. And it's going to have to be finely tuned. You don't start with questions like, “What's your financial status?” You start with, “What is your major problem at this time?” “Who's handling it?”

Richard Wassersug 20:12
We could talk about that later. But I think you've got my main main concerns.

Yuval Gal 20:16
Thank you, Richard. That's absolutely true. I mean, we have two kinds of potential users on our platform. It could be the caregiver in many cases. However, not everyone is fortunate enough to have a caregiver. So, at times, it will be the patient themselves. But you're absolutely correct. In a majority of cases, it will indeed be the caregiver. Also, pinpointing the patient's primary needs is a crucial aspect that we're considering. Your observation about this is indeed valid, and your insights are greatly appreciated.

Richard Wassersug 20:55
You can contact me if you want to extend the conversation strategically, I'd be happy to explore with you how to make this work.

Brad Power 21:06
I would like to cue Noel, who is a caregiver for her husband, Phil, and she is the representative of the persona that you identified. What would you say is unique about your perspective?

Noel Resch 21:33
What's unique about my perspective? You know, I tried to take a lot of the pressure off Phil, because he still works full time and wants to do that. My perspective is, I'm not the patient, I don't go through all the things he does. And while he's my spouse, and I think we're very close, I still don't really know how much he actually shares with me. I've been around enough other male cancer patients to indicate that they don't share absolutely everything with their spouse, they do hide a lot. So while I think I know him well on what he is going through, I don't know that I do. I guess that's the one thing I can say about caregiving that makes it difficult. I go in and I try to look, you know what, everything that's best, and I can give suggestions. But in the end, it all has to be Phil's decision. The ADT stuff. I don't know what that's like, I can hear about it. I

see him. But I'm pretty sure he goes through a lot more than what he shares with me. ADT is hard. And Phil's been on it for over five years now.

Brad Power 22:44
Noel, what is your regular job?

Noel Resch 22:51
I am a retired corrections officer. I am part of the community advisory board for UW (University of Wisconsin) Carbone Cancer Center in Madison. It's where we originally started as patients. So we have quarterly meetings. I am involved with that. I haven't gotten as involved as I would like to be yet COVID kind of slowed a lot of things down. So all of our meetings are still virtual. We haven't even really gotten together as a group. It's just been hard. They take people from all over the state of Wisconsin, so it's been hard to find a time when everyone can get together. But I do serve on that board. And I'm very active with PCF doing a lot of the fundraising and different activities.

Brad Power 23:42
For our Israeli friends, that's Prostate Cancer Foundation, which is the 800 pound gorilla in the prostate cancer space of research and funding.

Jeffrey Dwyer 24:09
I'm four years into this cancer journey. I'm 76 years old, so I'm an older guy with a terminal disease. Things are much different for younger men who are diagnosed with prostate cancer. I'm looking at you three guys, and you’re all young men. As you were talking, I kept thinking about what I would have wanted to know when I was diagnosed. There is so much to learn about the disease, and when the diagnosis presents itself, a person reflects and wishes they would have done things earlier in their lives to prepare for what is facing them with prostate cancer. I have a suggestion as you build-out your organization. I believe it would help if you had someone knowledgeable about long term disability care insurance. When people are young, they seldom think about being old and ill.

I have no idea about what is available in the marketplace today as far as long term disability insurance is concerned. I bought my policy 22 years ago, because I have always been an entrepreneur. I realized that there was never a safety net for me if I couldn’t work. I was not part of a larger organization. If I ran out of money, that was it.

So I had to have some means to pay the overhead to keep myself and my businesses functioning. I bought a long term care disability insurance policy. I paid the premiums for twenty-two years, and now I’m glad that I did. When my prostate cancer and cardiac diseases made it clear that the diseases were not curable, I made the claim to the long term disability insurer. The insurer went through their evaluation process and began reimbursing me for what I am spending for housekeeping, nursing, transportation and other expenses related to my various ailments..

Your company can probably partner with an independent insurance provider who is knowledgeable about the long term care disability insurance products that are available in the marketplace. This information will help the men who are diagnosed with the disease at a younger age more than those who are diagnosed later in life.

Today, the goal of prostate cancer care and really all diseases is to make the diseases chronic instead of death sentences. Obtaining long term care disability insurance like all insurance coverages involves an evaluation process by the insurers of the applicant’s health status. Since prostate cancer is most often a progressive disease, it would be wise for a man who is diagnosed with the disease to immediately apply for long term care disability insurance to determine if the insurance coverage is available to him for a reasonable cost. The insurer will evaluate the status of the applicant’s disease, and the cost of the insurance product will be priced accordingly.

Please counsel your clients not to lie on their insurance applications. Tell them to fully disclose their ailments so that their claims will not be denied at a future date when they need help paying their care bills. Men who are going to be on “active surveillance” may die not from the disease, but instead with it, but the quality of their lives may be affected by the disease and disability insurance may help them.

The other suggestion that I have is that your company counsel your clients to explore any benefits that they may be entitled to receive from the Veterans Administration if your clients served in their country’s military. Many veterans simply do not investigate the benefits that are offered from the VA. Until recently, I did not. I’m a veteran of service in the Navy, and now, after applying for coverage, the VA is supplying my hearing aids and eyeglasses for no cost. I have been assigned a PCP provider, and my medications will be paid for by the VA. I can select the private caregivers whom I want to use, and the VA will pay that cost.
Brian McCloskey 31:00
Would help navigating the VA have been helpful?

Jeffrey Dwyer 31:05
Yes, probably. Not availing myself of VA care earlier was probably both my combined ignorance and being macho. I thought that I didn't do much in the Navy except hiding for four years until I fulfilled my enlistment requirements. So, I just kind of felt, I should let the men who really contributed something use the VA care. Eventually I decided that I’d investigate what was available through the VA, and I was encouraged to use the benefits that had been put into place for all veterans. The VA partners with Medicare, Medicaid and private insurers to make sure that the country’s veterans are well provided for when they need help. I wish that I’d investigated what was available sooner. The American Veteran Administration system is this nation’s closest thing to national health care.

Brad Power 31:56
Jeff is giving you an insight into the unique characteristics of the American reimbursement system or insurance or health payments system.

Rick Stanton 32:19
I hope my hematocrit stays above nine. I'm excited that I got approved by America Genex so I'm hopping on this. evaluate them and tall. Thank you so much. I really like what you're doing. I like your vibe even more, because I know you're early in your journey. Thank you. I hope we can hear more.

Brian McCloskey 32:48
That's awesome news. This is another pioneering treatment, targeting B7-H3. So obviously, you're gonna want to keep us up to date. Maybe we even want to carve out a special section on on Circle for what you're experiencing. It takes a mental notes.

Rick Stanton 33:13
I hope my dramatic Thank you. It's not a done deal that, um, this will be a qualification visit to Why do you have to do labs again? I was like, just above what? You got to do.

Brian McCloskey 33:29
You fine. Yeah. Like positively doctors are vampires. Guys,

Yam Catzenelson 33:39
Yeah, absolutely. I just want to clarify something. You mentioned that you're not sure about the healthcare systems in Israel, Canada, and the UK. However, regarding Berries, we're planning to launch in the US and have been extensively researching the US healthcare system. So, we're here to support you in that context.

Brad Power 34:04
Rick is going for a biomarker that he identified through some of the kinds of exceptional testing that we have was identified through RNA seq. Is that correct, Brian?

Brian McCloskey 34:21
He has a very high expression of B7-H3 CD 276. The treatment is an antibody drug conjugate. It's cutting edge, for sure. But hopefully, he's a good candidate, because I think he's in the 90 Plus percentile across 1000 prostate cancer patients for B7-H3. So hopefully, this is going to help.

Brad Power 34:54
Reinforcing what Richard was saying, this is an exceptional group. These are people who are very much on the cutting edge of treatments, both diagnostics and treatments.

Robert Gurmankin 35:10
Just three things. One is another audience you'll need to look at is children of patients. I see a lot of them on the Facebook prostate cancer support group. Lots of mostly women asking questions about their dads. So that's a big target. The other thing is, I don't know if you've ever looked at it; there is a platform called Cake. Actually, it's www.joincake.com. And they do a lot of

stuff with end of life, things like documents, memorials, creating a will. And then the other thing they do, which I've done separately, is a digital will. So for all your online life, what to do with all that and how to handle that. You know, that's something that probably five or six years ago, not too many would think about, but it's important now. Thank you.

Paul Van Camp 36:58
My question is about navigating treatment decisions at all stages. I'm not sure if your business is addressing all cancers. In all persons, or just prostate, we're very prostate cancer specific. And that territory is hard enough. So a lot of men don't have optimal oncology teams in their decision making. Now a lot of men do not participate very actively and don't ask questions when they're either doctors, or they may be seeing the urologist that they've been seeing, since way back when, who's really not versed on this, the state of the art of prostate cancer therapy at various stages, and they're getting suboptimal treatment. And then again, as was mentioned a lot of times, it's a family member who's navigating, and inquiring about treatment decision points, and optimal choices. And so this requires, first of all, defining what's this standard of carers, as it's known what the established generally accepted best care as a baseline, and then these in our country in the US is pretty much embodied in the in the NCCN guidelines, for prostate cancer treatment and for other other cancers. And I don't know if there's an equivalent in England, in Europe and in Canada, but I know there are differences in which treatments can be accessed. So they'll have all of that. And it probably would be good to have a module or a service that can link people up with second opinions from now that telemedicine is more widely available, this is easier in many cases, than having to go get on an airplane and fly to some major Center of Excellence to speak with the best minds in medicine and in prostate cancer treatment. They have all of that.

But then you have another arena, which is clinical trials. How do you find clinical trials that are applicable to your particular case? It's a navigation problem. So search in gyms or maybe an AI bot that could take a person's history and state and what treatments they failed on. their genetic profile is, etc., and integrate that with clinical trials that may be accessible to them by geography and by criteria in their medical record, and spit out, okay, here's clinical trials that you may be eligible for, and you should consider. And then moving a step beyond that, non standard of care options, excellent treatments, possibly superior treatments that have not yet been integrated into the standard of care. And many here in this group are on non-standard care options, you can call them experimental therapies as I am. How do you navigate those possibilities and weigh them? You may not want to go into that. That's difficult, because you're talking about things that have risks. And the benefits have not yet been absolutely proved in clinical trials. And yet, as many of us have examples, we want to avail ourselves of the best possible treatments, weighing the risks, and knowing that the standard of care is an ever moving target. I call it the ragged edge of the standard of care. So how do you help people navigate their treatment decisions?

Yuval Gal 41:35
You've made some excellent points, and we're committed to addressing these issues. We're currently exploring how exactly to tackle them; aiding people in finding a reputable Second

Opinion is a concept we believe can offer significant value. Similarly, assisting them in locating suitable clinical trials aligns with our vision, as you mentioned. This reinforces our direction and enthusiasm for this aspect. Moreover, I'd like to highlight the issue of providing medical advice. We're actively working on understanding how to offer assistance without crossing into the territory of medical professionals. We have a genuine desire to contribute in this realm. However, it's important to be transparent that, at the moment, our primary focus is more in the operational sphere than the medical domain. Our initial emphasis is on managing tasks related to finances, applications for various plans, connecting you with nonprofits, and aiding in finding appropriate insurance coverage. This operational approach is our initial priority. The medical navigation you've brought up is indeed invaluable. I understand this firsthand, and I assure you, we are earnestly considering these aspects as we develop our platform.

Brad Power 43:29
We have a services guide with treatment options. Biomarker matching services is number 12. We've identified providers of those services, including clinical trials, like Massive Bio, whom we work with. As you said, you might outsource some. So we have something that you might start with. That's great if you're looking for that kind of outsourcing.

Brian McCloskey 44:11
Are you all familiar with Nancy's List? We had Nancy Novak speak with us. She was helping us understand ways to navigate the financial aspects of managing cancer. And she came in from the standpoint of, the cancer patient should not have to worry at all about any financial component of their care, but just take it off the table. And that was her mission. She's a cancer survivor. And so anyway, she came up with Nancy's List. And so I just put it in the chat, her website and then also the, the session that she had with us was back in December. So you might want to check that out.

Yuval Gal 45:46
Firstly, we want to express our gratitude for dedicating your time to assist us, share your experiences, and provide us with valuable insights. We will also be sharing a link, and if anyone is interested in meeting with us to chat or seek personalized guidance on how we can support them through their journey, we're more than open to that. It's a service we'd be thrilled to provide.

Richard Wassersug 46:36
I'm in Canada. So suddenly these things do not apply. I mean, we have a somewhat of a socialized medical system here. It's not nearly as good as the veterans hospitals in the United States. But it is actually better than the Canadian healthcare system. But, guys, feel free to contact me because there are strategic ways to get your product to the right audience. And that's a big issue. There's so many resources out there, you didn't mention this. But the average patient can look into YouTube and put “prostate cancer”. And by the time they've got a few 1000 hits, they just walk away in bewilderment. So the issue is going to be really targeting and finding the right audience to present to, not the product as much as getting the right place. I'd be happy

to talk to you about as well as for what questions to ask your doctors. I mean, this doesn't pertain to you. But I have strategies and gambits that I know work.

Richard Wassersug 47:47
That's pretty much the way I started. My learning process when I was diagnosed with prostate cancer was whatever I could find online.

David Plunkett 48:00
To the Berries group, I expect you're going to spend a lot of time examining what's already available, what's already out there. Try not to reinvent the wheel.

Brian McCloskey 48:16
You want to provide something that works better, but you don't need to reinvent the whole thing.

Yam Catzenelson 48:29
That would complement what you've just mentioned. We sincerely thank you for investing your time, and we genuinely wish you all the best. We hope to stay in touch and would be very interested in any opportunity to connect. Once again, thank you very much.